

1. Administrative & Study Identification

Full Study Title: A Phase I/II Open-label Dose Escalation and Dose Expansion Study to Evaluate the Safety, Pharmacokinetics, Pharmacodynamics, and Efficacy of AZD4360 in Adult Participants With Advanced Solid Tumours **Short Title/Acronym:** CONCLUDE / Study for AZD4360 in Participants With Advanced Solid Tumours **Protocol Number:** D8930C00001 **NCT Number:** NCT06921928 **Sponsor Name:** AstraZeneca **Investigational Product:** AZD4360 (CLDN18.2-targeting Antibody-Drug Conjugate / ADC) **Phase of Development:** Phase I/II **Medical Monitor:** AstraZeneca Clinical Study Information Center

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To investigate the safety and tolerability, and to determine the Maximum Tolerated Dose (MTD) and/or a Recommended Phase II Dose (RP2D) of AZD4360 in previously treated participants with advanced solid tumours, through the assessment of Dose Limiting Toxicities (DLTs), Adverse Events (AEs), and Severe Adverse Events (SAEs). **Secondary Objectives:** To evaluate the preliminary efficacy (Objective Response Rate [ORR], Complete Response [CR], Partial Response [PR], Progression-Free Survival [PFS], Overall Survival [OS], Duration of Response), pharmacokinetics (PK), immunogenicity, and pharmacodynamics of AZD4360.

2.2 Background & Rationale To evaluate the safety, pharmacokinetics, and antitumour activity of AZD4360, a TOP1 inhibitor antibody-drug conjugate targeting Claudin-18.2 (CLDN18.2), in adult participants with locally advanced or metastatic solid tumours selected for expression of CLDN18.2. There is a high unmet medical need for targeted therapies in previously treated patients with advanced Pancreatic Ductal Adenocarcinoma (PDAC), Gastric or Gastroesophageal Junction Cancer (G/GEJC), and Biliary Tract Cancer (BTC).

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (Module 1: AZD4360 monotherapy) **Blinding:** Open-label **Randomization:** N/A (Dose Escalation and Dose Expansion)

3.2 Planned Enrollment & Duration Target **NS:** 117 adult participants **Number of Sites:** Multi-centre global trial **Estimated Study Start:** 29-Apr-2025 **Estimated Primary Completion:** 31-Mar-2027

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Participant must be ≥ 18 years at the time of signing the ICF. 2. Eastern Cooperative Oncology Group (ECOG) performance status of 0-1 with no deterioration over the previous 2 weeks prior to baseline or day of first dosing. 3. Minimum life expectancy of 12 weeks in the opinion of the Investigator. 4. Histologically confirmed advanced or metastatic Pancreatic ductal adenocarcinoma (PDAC), Gastric or Gastroesophageal junction cancer (G/GEJC), and Biliary tract cancer (BTC) with documented positive CLDN18.2 expression. 5. Participants must have received at least one prior line of systemic therapy in the advanced/metastatic disease setting, and have at least one measurable lesion according to RECIST v1.1.

4.2 Exclusion Criteria 1. Human Epidermal Growth Factor Receptor 2 (HER2) positive (3+ by IHC or 2+ by IHC and positive by in situ hybridisation) or indeterminate G/GEJC participants. 2. Unstable or active peptic ulcer disease or digestive tract bleeding, or clinically significant ascites that require drainage. 3. Central nervous system (CNS) metastases or CNS pathology with spinal cord compression or high risk of paralysis. 4. History of non-infectious interstitial lung disease (ILD)/pneumonitis. 5. Known serologic status reflecting active hepatitis B or hepatitis C, known HIV infection that is not well controlled, or active tuberculosis infection.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Ascending doses of AZD4360 in the dose-escalation phase to determine MTD and RP2D, followed by the chosen dose in the expansion phase. **Route of Administration:** Intravenous infusion (standard for ADCs). **Duration of Treatment:** Continues until objective disease progression, unacceptable toxicity, or participant withdrawal.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for managing disease-related symptoms and expected ADC-related toxicities. **Prohibited:** Concurrent investigational agents, anticancer therapies, or prior CLDN18.2 directed therapy.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent, Medical History, Tumor tissue testing for CLDN18.2 expression, baseline RECIST assessment
Baseline	Day 1	Enrollment, First Dose, PK/PD sampling, Safety Labs	Interim Ongoing Safety monitoring (DLT evaluation), PK/PD tracking, RECIST v1.1

tumor assessments | | **End of Study** | At Discontinuation | Final safety evaluations, subsequent survival follow-up tracking for OS |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Determination of Maximum Tolerated Dose (MTD) and Recommended Phase II Dose (RP2D) through the assessment of Dose Limiting Toxicities (DLTs), Adverse Events (AEs), and Severe Adverse Events (SAEs). **Measurement Method:** Clinical evaluation and grading according to the Common Terminology Criteria for Adverse Events (CTCAE).

7.2 Secondary & Exploratory Endpoints * Objective Response Rate (ORR), Complete Response (CR), and Partial Response (PR) based on RECIST v1.1. * Progression-Free Survival (PFS) and Overall Survival (OS). * Duration of Response (DoR). * Pharmacokinetics: Volume of plasma clearance, and half-life of AZD4360 and total antibody.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring to capture DLTs, treatment-emergent AEs, and changes in laboratory parameters. Specific attention to ILD/pneumonitis. **Serious Adverse Events (SAEs):** Standard 24-hour reporting mechanisms for SAEs to AstraZeneca and regulatory authorities. **Data Monitoring Committee (DMC):** A Dose Escalation Committee (DEC) will review all safety and DLT data from each cohort before deciding on dose escalations.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Analysis Set (all enrolled participants who received ≥ 1 dose of AZD4360) and Efficacy Analysis Set (participants evaluable under RECIST v1.1). **Sample Size Justification:** $N = 117$, calculated based on standard Phase I dose-escalation modular design parameters to safely establish the MTD, with sufficient expansion cohorts to assess early efficacy signals. **Handling of Missing Data:** Standard oncology data handling; time-to-event endpoints (e.g., PFS, OS) will be right-censored at the date of last contact or new systemic anticancer therapy.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Regular clinical site monitoring visits (onsite and remote) by the sponsor to verify source data and ensure strict protocol and safety adherence. **Audit Trail:** Data captured via validated electronic Case Report Forms (eCRFs) with systematic data clarification and query resolution tracking.

11. Study Identifiers & Governance

Primary ID: D8930C00001 **Registry Number:** NCT06921928 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** Study for AZD4360 in Participants With Advanced Solid Tumours **Official Regulatory Title:** A Phase I/II Open-label Dose Escalation and Dose Expansion Study to Evaluate the Safety, Pharmacokinetics, Pharmacodynamics, and Efficacy of AZD4360 in Adult Participants With Advanced Solid Tumours

12. Clinical Framework (Solid Tumours Specific)

Primary Condition: Pancreatic ductal adenocarcinoma (PDAC), Gastric or Gastroesophageal junction cancer (G/GEJC), and Biliary tract cancer (BTC). **Diagnosis Threshold:** Histologically confirmed advanced or metastatic disease failing at least one prior line of systemic therapy. **Medical Methodology:** Targeted antibody-drug conjugate therapy (TOP1 inhibitor). **Optimization Target:** Targeting cells with documented positive Claudin-18.2 (CLDN18.2) expression.

13. Study Procedures & Methodology

Management Pathway: Standard oncology treatment pathways, adapting RECIST v1.1 for response tracking. **Education Protocol (HCP):** Site initiation and training on ADC safety profile, specifically monitoring/management of potential DLTs and pulmonary toxicities. **Education Protocol (Patient):** Comprehensive informed consent regarding early-phase investigational ADC therapy and strict contraception requirements. **Quality Audit Mechanism:** Ongoing safety audits by the sponsor and Dose Escalation Committee reviews.

14. Observation & Follow-Up Schedule

Total Duration: From screening until disease progression, death, or study completion (Estimated Study Completion Date: 16-Dec-2027). **Onsite Visit Cadence:** Regularly aligned with treatment cycles for intravenous drug administration and safety/PK sampling. **Remote Monitoring Frequency:** Routine survival follow-up phases post-discontinuation. **Checklist Review Interval:**

Radiological tumor response assessments per standard intervals (e.g., every 6-8 weeks) as defined by protocol.

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				